



Pain Management

Assessment

Pediatric Pearls:

- Use pediatric dosing of medications or electrical therapy for a pediatric patient < 37 kg and as defined by HandTevy.

Signs & Symptoms:

- Severity (Pain scale)
- Quality
- Radiation
- Relation to movement
- Respirations
- Reproduceable
- Increased upon palpation

Differential:

- Per the specific protocol
- Musculoskeletal
- Visceral (abdominal)
- Cardiac
- Pleural / Respiratory
- Neurogenic
- Kidney stone

Clinical Management Options

P	P	P	P	P	P	• Oxygen
L	L	L	L	L	L	• Pain scale assessment 0-10, Wong-Baker faces for pediatrics, FLACC for infants
1	2	3	4	5	6	• Splinting as needed • Ice pack as needed • Bilateral BP measurements • SPO2 monitoring
						• ECG application • ETCO2 monitoring • Acetaminophen – Adult PO only • Ibuprofen – Adult PO only
						• Acetaminophen • Vascular access • Isotonic Crystalloid as needed
						• Monitoring and interpretation of ECG & EtCO₂ • Fentanyl • Ketamine only if respiratory failure; or pain unchanged by Fentanyl • Lidocaine for suspected kidney stone

Consult Online Medical Control As Needed

Pearls:

- Refer to drug formulary charts for all medication dosing for both adults and pediatric patients.
- Pain severity is a vital sign to be recorded pre and post pain intervention, especially medications.
- Vital signs should be obtained pre and 5-minutes post all medications.
- Monitor patient closely for over sedation, refer to Overdose COG if needed
- Sedating medications should be administered cautiously in head injury patients to avoid obscuring mental status exam
- Do not administer Acetaminophen to patients with history of liver disease or known to have consumed large amounts of ETOH.
- Fentanyl and Ketamine should be reserved for acute pain.
- Abdominal aneurysms may present as back pain and are a concern in patients over the age of 50.
- Any new bowel or bladder incontinence is a significant finding which requires immediate medical evaluation.
- In patient with history of IV drug abuse or pain management injections, an epidural abscess should be considered.
- Controlled substances are discouraged for non-traumatic back pain.
- Sedating medications should be administered with caution in patients already taking sedating medications